



The Belgian Cancer Inventory: Co-creating a framework for monitoring the implementation of cancer policies and associated inequalities



Robbe Saesen^{a,*}, Gabrielle Schittecatte^{a,1}, Jinane Ghattas^a, Marie Delnord^a

^a Belgian Cancer Centre, Sciensano, Brussels, Belgium

Background: In 2021, the European Commission launched the policy-driven Europe's Beating Cancer Plan [1], and the research-focused Mission on Cancer [2]. These initiatives recognise the burden cancer poses on society, and offer opportunities to reduce this burden through targeted actions. However, the uptake of those actions is not systematically monitored in every Member State. In Belgium, a country lacking an up-to-date national cancer plan, a monitoring and evaluation framework for cancer care and control policies is currently under development at the request of the federal cabinet, covering both Belgian- and European-level measures. This framework, known as the Belgian Cancer Inventory, will also aim to identify inequalities in the implementation and outcomes of each policy it tracks.

Methods: To ensure the Inventory meets the needs and expectations of stakeholders in the Belgian cancer field, a participatory, co-creation approach was leveraged for its design. Through a Delphi exercise, experts from across the cancer care continuum were asked to assess over the course of three rounds the importance and actionability of a set of 259 indicators extracted from grey and scientific literature, as well as of seven inequality dimensions taken from the European Cancer Inequalities Registry [3] by which these indicators could be disaggregated (e.g. age, income level, region). Based on pre-defined consensus criteria, advancement to the next round was determined.

Results: In total, the Delphi study featured contributions from 94 participants. Following the first round during which importance was the sole evaluation criterion, all indicators and dimensions were retained, barring one survivorship-related indicator. Furthermore, 74 indicators and 19 dimensions were added at the suggestion of the panellists. At the

end of the second round in which both actionability and importance were evaluated, 72 indicators and five dimensions were eliminated. In the third round, contributors ranked the remaining Delphi items on the basis of priority for inclusion into the Inventory. Ultimately, the top-ranking indicators were matched to Belgian and European policy objectives, and six inequality dimensions were selected: age, sex, educational level, objective and self-reported income level, and region.

Conclusions: Using a co-creation method, an initial framework for the Belgian Cancer Inventory was developed, including indicators and integrating inequality dimensions. While this framework presents a promising start to the systematic monitoring of cancer policies and associated inequalities in Belgium, it will need to be further refined through a feasibility analysis and benchmarking exercise. Questions remain on the completeness of the data that would feed into the Inventory.

References

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* Corresponding author.

¹ Shared first authors.